

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

COLD CALL OPENING SCRIPT

Field-Ready Conversation Guides for Hospice Sales

Complete scripts, discovery questions, objection rebuttals, and conversation branches for new account development.

Cold Call Opening Script

The Spartan Approach to First Conversations

THE HEALTHCARE SALES MASTERY MODEL

Every successful hospice sales conversation follows a four-stage progression. Understanding which stage you are in determines how you speak, what you ask, and what you offer next. Rushing through stages is the single most common reason deals fall apart. Train yourself to slow down.

1**DISCOVERY**

Learning about the operations, processes, and organizational priorities of the account

2**CONNECTING**

Building individual rapport and understanding the personal motivations of your contact

3**GUIDING**

Aligning the problems they have shared with the solutions you can provide

4**COMMITMENT**

Asking for the referral or partnership with confidence and clarity

THE SPARTAN 30-SECOND OPENER

Your opening sets the entire tone of the relationship. Most hospice reps lead with their company name and credentials. Spartan-trained reps lead with the patient. The following opener works because it acknowledges their time constraints, centers the conversation on outcomes, and asks for minimal commitment upfront.

"Hi [Name], this is [Your Name] with [Hospice Company]. I know you are dealing with a full caseload right now, so I will keep this brief. We partner with facilities like yours to help ensure patients with serious illness get into comfort-focused care at the right time instead of the last minute. I would love to ask you two questions about how your team currently approaches those decisions. Do you have 90 seconds?"

Why This Opening Works

- Opening with acknowledgment of their workload builds immediate rapport and reduces defensiveness

- Framing hospice around timing and outcomes rather than services positions you as a clinical partner
- Asking for 90 seconds instead of a meeting lowers the perceived cost of saying yes
- Asking for two questions rather than presenting creates a dialogue rather than a pitch

THREE CONVERSATION BRANCHES

Branch 1: They Say Yes

When they agree to talk, move immediately into Stage 1 discovery without any preamble. Do not thank them excessively or repitch your company. Get into the questions.

"Perfect. First question: when a patient in your facility reaches a point where curative treatment is no longer helping, what typically happens with their care plan from your team's perspective?"

Listen fully. Do not interrupt. After they answer, follow with:

"That is helpful. And how frequently does your team connect with a hospice liaison to help think through those decisions together?"

These two questions tell you everything: whether they have a process, how often they use it, and whether there is a gap you can fill. From there, you can book a follow-up meeting or offer an educational resource based on what they share.

Branch 2: They Are Resistant or Rushed

Resistance is not rejection. It is usually a time or priority issue. If they say they are busy or not interested, do not argue. Pivot to a low-commitment ask.

"Absolutely, I understand. I am not here to take up your time. Would it be okay if I dropped off a short resource next week when you have a quiet moment? It takes about two minutes to read and covers the eligibility criteria most facilities find helpful for their more complex patients."

This keeps the door open and positions your next visit as a value delivery rather than a sales call. The resource gives them a reason to see you again.

Branch 3: The Gatekeeper Stops You

Front desk staff and nurses who screen calls are not obstacles. They are potential champions. Treat every gatekeeper with the same respect you would give the decision-maker.

"Hi, my name is [Your Name] from [Hospice Company]. I work with your social work team and nursing leadership on making sure patients with serious illness get evaluated at the right time. I am not here to sell anything today. I am just looking to introduce myself and learn about how your team works. Would [name] have five minutes sometime this week?"

Ask the gatekeeper for their name. Use it. Ask them what the best time of day is to catch the decision-maker. They will often tell you exactly when and how to reach the right person.

FIVE DISCOVERY QUESTIONS WITH PURPOSE EXPLANATIONS

These questions are designed to open up a genuine conversation in Stage 1. Never fire more than two or three in one visit. They should feel like natural curiosity, not a structured interview.

1 The Census Question

"How many patients in your facility right now would you estimate are appropriate for hospice comfort care?" —

Purpose: This immediately establishes shared clinical ground and often reveals that they have not thought about this. Most facilities underestimate. When they answer, it opens the door to a discussion about identification processes.

2 The Process Question

"What does your current process look like when a clinician identifies a patient who might benefit from hospice?" —

Purpose: This reveals whether there is a formal or informal process, who is involved, and where the bottlenecks are. The answer tells you exactly where your support would be most valuable.

3 The Timing Question

"When patients do get referred to hospice, how long are they typically on service before they pass away?" — Purpose: This opens a conversation about late referrals, which is one of the biggest issues in the field. Most facilities know their average length of stay is shorter than ideal. You are not telling them they are doing it wrong. You are asking them to acknowledge it themselves.

4 The Barrier Question

"What gets in the way of earlier conversations with patients and families about comfort-focused care?" — Purpose: This question invites them to name their pain point. They might say family resistance, physician hesitation, or staff discomfort. Whatever they say, it is your next educational topic.

5 The Relationship Question

"Who else in your building is typically part of those care transition conversations?" — Purpose: This is your stakeholder mapping question. You are identifying who else you need to meet and build a relationship with before you can advance the partnership.

FOUR OBJECTION REBUTTALS

Objection 1: "We already have a hospice we work with."

"I respect that. A lot of the facilities I partner with work with multiple hospice providers depending on the patient's location, insurance, or family preference. I am not here to replace anyone. I am just hoping to learn about your experience and see if there is a way we can support your team in specific situations where your current partner might not be the right fit. Would it be helpful to at least know we are available?"

Objection 2: "We are not taking new referral partners right now."

"That makes complete sense. I would not want to add anything to your plate. What I would ask is whether I could simply introduce myself and drop off a clinical resource on eligibility criteria. No relationship required, no commitment. If something changes down the road and you ever need a second option, I want you to know my name and number."

Objection 3: "We handle our own discharge planning."

"That is great to hear and it shows how much your team cares about continuity. My role is not to interfere with that process. What I find most valuable is when I can partner with your discharge planners to make their job easier for the most complex cases. Would it be worth a 15-minute conversation with your discharge planning lead just to see if there are any gaps I might be able to help fill?"

Objection 4: "I do not have time for this right now."

"Completely understood. I will be brief. Could I simply leave my card and one resource and follow up by email? The resource has practical information about hospice eligibility criteria for your three most common diagnoses. No strings attached. If it is useful, great. If not, no harm done."

SPARTAN FIELD PRINCIPLE

Every objection is a doorway. Behind "we already have a hospice" is often a dissatisfied referral source who has never been asked how things are actually going. Behind "not interested" is often a person who has been burned by pushy reps before. Your job is to be so different from what they expect that they cannot help but give you five minutes.